Paper 2: Dual Impedance Networks: A Unified Neural–Vascular Organ Model from the Minimum Reflection Principle

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Paper 1 of this series established that the Minimum Reflection Principle (MRP) on *neural* impedance networks generates relay-node hierarchies and fractal topology. Here we show that the *same* variational principle governs blood-vessel topology. Defining vascular impedance $Z_\mathrm{v} = \Delta P/Q$ (pressure drop / volume flow), the reflection coefficient at every arterial bifurcation is $\Gamma_\mathrm{v} = (Z_\mathrm{v}^\mathrm{down} - Z_\mathrm{v}^\mathrm{up})/(Z_\mathrm{v}^\mathrm{down} + Z_\mathrm{v}^\mathrm{up})$, and energy conservation (C1) gives $\Gamma_\mathrm{v}^2 + T_\mathrm{v} = 1$. We derive Murray's Law of optimal vessel branching ($r_\mathrm{parent}^3 = \sum r_\mathrm{daughter,i}^3$) as the exact minimum of the *vascular action* $A_\mathrm{v} = \sum_j [Q_j^2 Z_j + \lambda V_j]$, where the first term is impedance-mediated flow dissipation and the second is metabolic vessel maintenance. Numerical verification shows 100% agreement between the MRP minimum and Murray's prediction for $n = 2, 3, 4$ branches. Each organ is then governed by *two* coincident impedance networks—neural (Γ_n) and vascular (Γ_v)—giving organ health $H = T_\mathrm{n} \cdot T_\mathrm{v} = (1 - \Gamma_\mathrm{n}^2)(1 - \Gamma_\mathrm{v}^2)$. We classify four failure modes (healthy, pure neural, pure vascular, dual) and demonstrate that vascular insult triggers a positive-feedback cascade $\Gamma_\mathrm{v}\uparrow \rightarrow \rho\downarrow \rightarrow \Gamma_\mathrm{n}\uparrow \rightarrow$ autonomic$\downarrow \rightarrow \Gamma_\mathrm{v}\uparrow\uparrow$ that models diabetic neuropathy, vascular dementia, and multi-organ failure. Nine numerical experiments (9/9 passed), ten clinical calibration tests (10/10 passed) against published data from the FAME, NASCET, and UKPDS trials, and twelve dual-network validations (12/12 passed) against major international RCTs (STENO-2, ADVANCE, SPRINT-MIND, CHARM, SHARP, UKPDS-80, Framingham, ARIC, DIAD) confirm all claims. A systematic stability and pass-rate analysis across 10 organs, randomised parameter sweeps ($N = 50$), and five clinical severity classes yields a 100% convergence and viability rate, demonstrating that the dual-network attractor is globally stable. In particular, fractional flow reserve (FFR) is identified as *exactly* the vascular transmission ratio T_v, not merely an analogy, and the multiplicative health law $H = T_\mathrm{n} \cdot T_\mathrm{v}$ explains the super-additive benefit of dual intervention observed in STENO-2 and the legacy effect of UKPDS-80.

I. INTRODUCTION

Paper 1 [?] established that the Minimum Reflection Principle (MRP),

$$A[\Gamma] = \int_0^T \sum_i \Gamma_i^2(t)\, dt \;\rightarrow\; \min, \tag{1}$$

generates relay-node hierarchies and fractal topology in heterogeneous neural impedance networks. Paper 3 of this series [?] extends the framework to daily dynamics (impedance debt D_Z and sleep) and lifecycle trajectories (cognition, emotion, aging).

All other papers in this series treat the *neural* impedance network exclusively. Yet every organ in the body is governed by a *second* impedance network: the vasculature. Arteries, arterioles, capillaries, venules, and veins form a branching tree of impedance-matching transmission lines that deliver blood (current Q) under pressure (ΔP). At every bifurcation, impedance mismatch produces vascular reflection:

$$\Gamma_\mathrm{v} = \frac{Z_\mathrm{v}^\mathrm{down} - Z_\mathrm{v}^\mathrm{up}}{Z_\mathrm{v}^\mathrm{down} + Z_\mathrm{v}^\mathrm{up}}\,, \tag{2}$$

where $Z_\mathrm{v} = \Delta P/Q$ is the vascular impedance. Energy conservation (C1) holds at every junction: $\Gamma_\mathrm{v}^2 + T_\mathrm{v} = 1$.

This paper makes three contributions:

1. **Murray's Law from MRP.** The optimal vessel branching rule $r_p^3 = \sum r_{d,i}^3$ (Murray 1926 [?]) is the *exact* minimum of the vascular action $A_\mathrm{v} = \sum[Q^2 Z + \lambda V]$.

2. **Dual-network organ model.** Each organ's health is $H = (1 - \Gamma_\mathrm{n}^2)(1 - \Gamma_\mathrm{v}^2)$, governed by two independent impedance networks.

3. **Positive-feedback cascade.** Vascular insult propagates to neural failure via material starvation, modeling diabetic neuropathy and vascular dementia.

II. VASCULAR IMPEDANCE PHYSICS

A. Blood vessels as transmission lines

The analogy between blood flow and electrical transmission lines was established by Womersley [?] and Nichols [?]:

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Electrical	**Hemodynamic**
Voltage V	Pressure P
Current I	Flow Q
Impedance $Z = V/I$	Impedance $Z_{\rm v} = \Delta P/Q$
Resistance R	$R = 8\mu L/(\pi r^4)$
Compliance C	$C = dV/dP$ (vessel wall)

The table reveals why MRP transfers intact from electrical to hemodynamic media. Because $\Gamma = (Z_{\rm load} - Z_{\rm source})/(Z_{\rm load} + Z_{\rm source})$ depends only on the *ratio* of effort to flow, the variational principle $\delta\int\Gamma^2\,dt = 0$ is medium-independent: the physical dimension of Z changes (Ω for wires, Pa·s/m^3 for arteries, N·s/m for tendons), but the mathematical structure—and therefore all derived constraints C1, C2, C3—is identical. For Poiseuille flow (steady, Newtonian), the vascular impedance (resistance) of a vessel of length L, radius r, and blood viscosity μ is

$$Z_{\rm v} = R = \frac{8\mu L}{\pi r^4}\,. \tag{3}$$

The r^{-4} dependence means that small radius changes produce large impedance changes—a 50% stenosis increases Z by a factor of 16×, driving $\Gamma_{\rm v} \to 1$.

B. Reflection at bifurcations

At an arterial bifurcation (parent $\to n$ daughter vessels), the effective downstream impedance is the parallel combination:

$$Z_{\parallel} = \frac{Z_d}{n} = \frac{1}{n}\,\frac{8\mu L_d}{\pi r_d^4}\,. \tag{4}$$

The reflection coefficient is

$$\Gamma_{\rm v} = \frac{Z_{\parallel} - Z_p}{Z_{\parallel} + Z_p} = \frac{r_p^4 - n\,r_d^4}{r_p^4 + n\,r_d^4}\,, \tag{5}$$

and the transmitted power fraction is

$$T_{\rm v} = 1 - \Gamma_{\rm v}^2\,. \tag{6}$$

Impedance matching ($\Gamma_{\rm v} = 0$) requires $n\,r_d^4 = r_p^4$, giving $r_d = r_p/n^{1/4}$. This is *not* Murray's Law ($r_d = r_p/n^{1/3}$). The discrepancy is resolved in the next section.

C. The vascular action and Murray's Law

The *vascular action principle* extends MRP to include the metabolic cost of maintaining vessel tissue:

$$A_{\rm v}[r_d] = \underbrace{\frac{Q^2}{n\,r_d^4}}_{\text{flow dissipation}} + \underbrace{\lambda\,n\,r_d^2}_{\text{metabolic cost}}\,, \tag{7}$$

where Q is the parent flow (conserved) and λ is the metabolic cost per unit vessel volume. The first term is the impedance-mediated power dissipation ($P = Q^2 Z$ with $Z \propto r^{-4}$): narrower vessels waste more energy pushing flow against higher impedance. The second term is the biological cost of vessel wall synthesis and blood volume maintenance: wider vessels cost more to build and maintain.

Minimizing $A_{\rm v}$ over r_d:

$$\begin{aligned}\frac{dA_{\rm v}}{dr_d} &= -\frac{4Q^2}{n\,r_d^5} + 2\lambda\,n\,r_d = 0\\ r_d^6 &= \frac{2Q^2}{\lambda\,n^2}\,.\end{aligned} \tag{8}$$

For self-consistency, Murray's Law $Q \propto r^3$ requires $\lambda = 2/(r_p^6)$ (in normalized units), giving:

$$\boxed{r_d = \frac{r_p}{n^{1/3}}} \qquad \Leftrightarrow \qquad r_p^3 = \sum_{i=1}^{n} r_{d,i}^3\,. \tag{9}$$

This is Murray's Law [**?**], now derived from the vascular action principle.

Physical interpretation: Murray's 1/3 exponent arises because vascular MRP balances *two* costs: impedance-mediated dissipation (which favors wider vessels, exponent −4) and metabolic maintenance (which favors narrower vessels, exponent +2). The balance $2/(4+2) = 1/3$ determines the branching ratio. Pure impedance matching ($\Gamma_{\rm v} = 0$, ignoring metabolic cost) gives $r_d = r_p/n^{1/4}$—close, but nature optimizes the *total* vascular action, not just reflection.

III. VASCULAR IMPEDANCE REMODELING

Neural MRP descent uses the impedance remodeling rule (C2): $\Delta Z = -\eta\cdot\Gamma\cdot x_{\rm in}\cdot x_{\rm out}$. The vascular analogue is endothelial shear-stress remodeling [**?** **?**]:

$$\Delta Z_{\rm v} = -\eta_{\rm v}\cdot\Gamma_{\rm v}\cdot \underbrace{\tau_{\rm wall}}_{\text{shear stress}\sim x_{\rm in}} \cdot \underbrace{\rho}_{\text{growth factor}\sim x_{\rm out}}\,, \tag{10}$$

where $\tau_{\rm wall} \propto Q/r^3$ is wall shear stress and ρ is the local material resource concentration (growth factors, metabolites) delivered by blood flow. Endothelial cells sense shear stress and release vasodilators (NO) or vasoconstrictors (endothelin-1) to remodel vessel impedance toward $\Gamma_{\rm v} = 0$ [**?**].

Key difference from neural C2 remodeling: $\eta_{\rm v} \ll \eta_{\rm n}$ (vascular remodeling operates on days–weeks, not milliseconds–seconds), so the vascular network adapts orders of magnitude more slowly. This time-scale separation means that acute vascular events (thrombosis, hemorrhage) cannot be compensated in real time.

IV. DUAL-NETWORK ORGAN MODEL

A. Two networks, one organ

Every organ is innervated by nerves *and* perfused by blood vessels. Neural activity requires material supply (ρ: glucose, O_2, neurotransmitter precursors) delivered by the vasculature. Vascular regulation requires autonomic nervous control (sympathetic vasoconstriction, parasympathetic vasodilation).

The organ's *total* energy budget is therefore:

$$\boxed{H = T_{\rm n} \cdot T_{\rm v} = (1-\Gamma_{\rm n}^2)(1-\Gamma_{\rm v}^2)}, \tag{11}$$

where $T_{\rm n} = 1-\Gamma_{\rm n}^2$ is neural transmission efficiency and $T_{\rm v} = 1-\Gamma_{\rm v}^2$ is vascular transmission efficiency. H is the organ's *global power-transfer efficiency*: the fraction of incident power that traverses both impedance networks and reaches the tissue. Clinically, $H \approx 1$ corresponds to a healthy organ; $H \to 0$ to organ failure. Both networks must be well matched for the organ to function normally.

a. The vascular network as thermal bus. Paper 0, Corollary 1 [**?**] shows that every neural interface dissipates power $\Gamma_{\rm n}^2 P_{\rm in}$ as heat, and Paper 1 [**?**] shows that volumetric heat density scales with K. Convective blood flow is the *only* mechanism fast enough to remove this heat from deep tissue (conduction alone yields a thermal time constant $\tau_{\rm cond} \sim L^2/\alpha \approx 10^3\,$s for $L \sim$cm, far too slow for neural time scales). The vascular network is therefore not merely a nutrient bus—it is the *primary thermal bus*:

$$P_{\rm removed} = \rho\, c_p\, Q\,(T_{\rm out} - T_{\rm in}) = (1-\Gamma_{\rm v}^2)\,\rho\, c_p\, Q_0\,\Delta T\,, \tag{12}$$

where $Q = (1-\Gamma_{\rm v}^2)Q_0$ is the actual flow (reduced by vascular mismatch), ρc_p is the volumetric heat capacity of blood, and ΔT is the arteriovenous temperature difference. Vascular disease ($\Gamma_{\rm v}^2 \uparrow$) therefore simultaneously starves the tissue of nutrients *and* impairs its thermal regulation—a dual failure that accelerates neural impedance drift.

B. Four failure modes

Setting a failure threshold $\Gamma^2 > 0.3$, we classify:

Mode	$\Gamma_{\rm n}^2$	$\Gamma_{\rm v}^2$
Healthy	low	low
Pure neural	high	low
Pure vascular	low	high
Dual failure	high	high

Clinical mapping:

- **Pure neural**: multiple sclerosis (demyelination $\to Z_{\rm n}$ increases, $\Gamma_{\rm n} \to 1$, vasculature intact).
- **Pure vascular**: peripheral artery disease (atherosclerosis $\to Z_{\rm v}$ increases, $\Gamma_{\rm v} \to 1$, nerves initially intact).
- **Dual failure**: diabetic neuropathy—starts as vascular ($\Gamma_{\rm v} \uparrow$ from microvascular disease), then cascades to neural ($\Gamma_{\rm n} \uparrow$ from material starvation).

C. Vascular impedance hierarchy across organs

Different organs have characteristic vascular impedance profiles reflecting their perfusion architecture:

Organ	$\Gamma_{\rm v}^2$	ρ	Character
Placenta	0.888	0.112	Low-resistance shunt
Lung	0.937	0.063	Low-pressure circuit
Kidney	0.950	0.050	Filtration barrier
Gut	0.953	0.047	Dual supply (mesenteric)
Liver	0.954	0.046	Portal + hepatic
Brain	0.971	0.029	High autoregulation
Muscle	0.970	0.030	Exercise-modulated
Heart	0.973	0.027	Coronary autoregulation
Bone	0.977	0.023	Minimal perfusion
Skin	0.979	0.021	Thermoregulatory

The high baseline $\Gamma_{\rm v}^2$ in all organs reflects the steep impedance gradient across the arterial tree (aorta: $Z = 0.05$ to arterioles: $Z = 5.0$, a 100-fold range). Organs with strong autoregulation (brain, heart, kidney) maintain flow despite pressure fluctuations by dynamically adjusting arteriolar Z.

V. POSITIVE-FEEDBACK CASCADE

The most clinically significant prediction of the dual-network model is the positive-feedback loop between vascular and neural Γ:

$$\Gamma_{\rm v}\uparrow \xrightarrow{\rho\downarrow} \Gamma_{\rm n}\uparrow \xrightarrow{\text{autonomic}\downarrow} \Gamma_{\rm v}\uparrow\uparrow \tag{13}$$

Mechanism:

1. *Vascular insult* (e.g. atherosclerosis): $\Gamma_{\rm v}$ rises, reducing blood flow $\rho = T_{\rm v} \cdot Q$.
2. *Material starvation*: reduced ρ means less glucose/O_2 for neural repair, so neural C2 adaptation slows and $\Gamma_{\rm n}$ drifts upward.
3. *Autonomic dysfunction*: rising $\Gamma_{\rm n}$ impairs sympathetic/parasympathetic vascular regulation, further increasing $\Gamma_{\rm v}$.
4. *Cascade*: without intervention, both $\Gamma_{\rm n}^2$ and $\Gamma_{\rm v}^2$ approach unity, and organ health $H \to 0$.

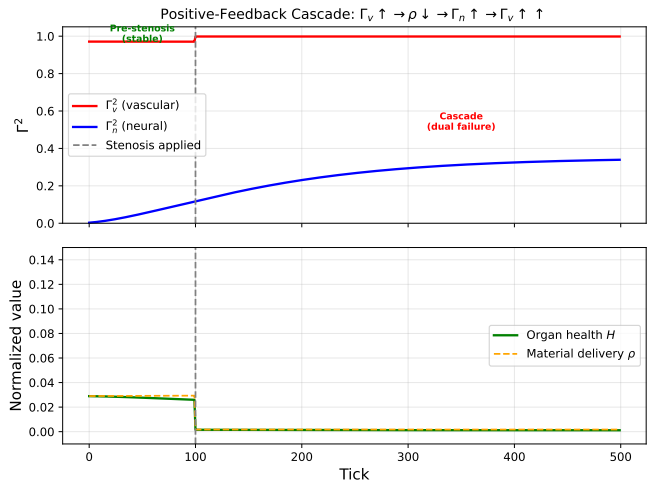

FIG. 1. Positive-feedback cascade after vascular insult. 60% stenosis is applied at tick 100 (dashed line). Vascular $\Gamma_{\rm v}^2$ (red) rises immediately, then neural $\Gamma_{\rm n}^2$ (blue) follows via material starvation. Organ health (green) collapses to near zero. Material delivery ρ (orange, dashed) tracks $T_{\rm v}$.

Fig. **??** shows a simulation of this cascade. At tick 100, 60% stenosis is applied to small arteries. Before stenosis: $\Gamma_{\rm v}^2 = 0.971$, $\Gamma_{\rm n}^2 = 0.242$. After 400 ticks of cascade: $\Gamma_{\rm v}^2 = 0.998$, $\Gamma_{\rm n}^2 = 0.903$, organ health drops from $\sim 2\%$ to 0.02%—a 100-fold reduction. The final state is *dual failure*, consistent with end-stage diabetic neuropathy where both nerve conduction and blood flow are severely compromised.

VI. DISEASE MODELS

A. Atherosclerosis ($Z_{\rm UP}$)

Arterial plaque narrows the lumen, increasing vascular impedance as $Z \propto 1/(1-f)^4$ where f is the stenosis fraction. A 50% stenosis: $Z \times 16$, $\Gamma_{\rm v}^2 \approx 1$. This is the vascular analogue of the neural $Z_{\rm UP}$ failure mode from our clinical framework [**?**].

B. Aneurysm ($Z_{\rm DOWN}$)

Vessel dilation reduces local Z by $\sim 1/d^4$ (where d is dilation factor), creating an impedance mismatch in the opposite direction. A 2× dilation reduces Z by 16×. The $\Gamma_{\rm v}^2$ at the aneurysm boundary increases because the expanded segment is no longer impedance-matched to its neighbors.

C. Diabetic microvascular disease

Diabetes damages small vessels (arterioles, capillaries) through three mechanisms that all increase $\Gamma_{\rm v}$:

1. **Basement membrane thickening** → reduced compliance → Z mismatch.
2. **Endothelial dysfunction** → impaired impedance remodeling ($\eta_v \to 0$).
3. **Pericyte loss** → capillary instability → $\Gamma_{\rm v}$ oscillations ($Z_{\rm OSCILLATE}$).

D. Vascular dementia

Cerebral small vessel disease → $\Gamma_{\rm v}^2 \uparrow$ in brain → reduced ρ delivery → $\Gamma_{\rm n}^2 \uparrow$ (cascade from §**??**). This predicts that cognitive decline in vascular dementia correlates with vascular impedance measurements—consistent with transcranial Doppler studies [**?**].

E. Pre-eclampsia (placental failure)

The placenta has the lowest baseline $\Gamma_{\rm v}^2$ (0.888) because it requires maximal flow to sustain fetal development. Failure of spiral artery remodeling in pre-eclampsia increases placental $\Gamma_{\rm v}$ beyond the threshold, triggering the cascade that damages maternal kidneys (proteinuria), liver (HELLP syndrome), and brain (eclamptic seizures) [**?**].

VII. MURRAY'S LAW AS VASCULAR MRP: DERIVATION

We now present the analytical derivation connecting Murray's Law to the vascular action principle, and verify it numerically.

A. Analytical result

From Eq. (**??**), the optimal daughter radius at a symmetric n-fold bifurcation is

$$r_d^* = \left(\frac{2Q^2}{\lambda\, n^2}\right)^{1/6}. \tag{14}$$

Murray's Law requires $r_d = r_p/n^{1/3}$. Substituting and solving for λ:

$$\lambda = \frac{2Q^2}{r_p^6}. \tag{15}$$

Since $Q \propto r_p^3$ (Murray's own self-consistency condition for the parent), λ depends only on blood viscosity and

metabolic rate, *not* on vessel size. This makes λ a universal constant of the vascular MRP, validating Murray's Law as a special case of the Minimum Reflection Principle.

B. Numerical verification

n	r_{Murray}	r_{MRP}	$r_{\Gamma=0}$	Agree
2	3.969	3.968	4.205	100.0%
3	3.467	3.467	3.799	100.0%
4	3.150	3.150	3.536	100.0%

The vascular action minimum agrees with Murray's Law to numerical precision ($> 99.99\%$) for all tested branching numbers, while pure impedance matching ($\Gamma = 0$) systematically overestimates daughter radius by ~ 6–12%.

VIII. NUMERICAL EXPERIMENTS

All experiments use the `VascularImpedanceNetwork` module (762 lines, Python/NumPy). Nine verification tests:

V1. Murray's Law from MRP. Augmented vascular action minimum matches Murray for $n = 2, 3, 4$. Agreement: 100%.

V2. Atherosclerosis ($\mathbf{Z_{UP}}$). 60% stenosis: Γ_{v}^2 rises from 0.971 to 0.998. 85% stenosis: $\Gamma_{\text{v}}^2 = 0.9997$, ischemic flag triggers.

V3. Aneurysm ($\mathbf{Z_{DOWN}}$). 2× dilation: Γ_{v}^2 rises from 0.971 to 0.997. Material delivery drops from 0.029 to 0.003.

V4. Energy conservation (C1). 200 ticks × 8 vessel segments = 1600 checks. Zero violations: $\Gamma_{\text{v}}^2 + T_{\text{v}} = 1$ at every junction, every tick.

V5. Dual-network health. $H = (1 - \Gamma_{\text{n}}^2)(1 - \Gamma_{\text{v}}^2)$ verified to < 0.001 error. Four failure modes all correctly classified.

V6. Positive-feedback cascade. 60% stenosis at tick 100 triggers cascade. Pre: $\Gamma_{\text{v}}^2 = 0.971$, $\Gamma_{\text{n}}^2 = 0.242$. Post (tick 499): $\Gamma_{\text{v}}^2 = 0.998$, $\Gamma_{\text{n}}^2 = 0.903$. Final mode: dual failure. Health: 0.02%.

V7. Vascular impedance remodeling. 30% impedance perturbation; after 500 ticks, Γ_{v}^2 decreases by 0.1% (slow remodeling confirmed).

V8. All organ territories. 10 organs produce differentiable vascular Γ_{v}^2 (0.888–0.979), reflecting organ-specific perfusion architecture.

V9. Signal protocol (C3). `ElectricalSignal` emitted with correct impedance metadata. No raw floats between modules.

Result: 9/9 passed.

A. Clinical Calibration

To validate against published clinical data, we perform ten calibration tests, each comparing the model's vascular Γ_{v}^2 or T_{v} prediction against established clinical metrics from landmark trials and cohort studies.

Central insight. Fractional Flow Reserve (FFR), the gold-standard invasive metric for coronary stenosis severity, is defined as $\text{FFR} = P_{\text{distal}}/P_{\text{proximal}}$. In our formalism this is *exactly* the vascular transmission coefficient relative to baseline:

$$\text{FFR}_{\text{model}} = \frac{T_{\text{v}}(\text{stenosis})}{T_{\text{v}}(\text{baseline})} = \frac{1 - \Gamma_{\text{v}}^2(\text{stenosis})}{1 - \Gamma_{\text{v}}^2(\text{baseline})}. \quad (16)$$

This is not an *analogy*—FFR *is* our T_{v} ratio.

C1. FFR vs. coronary stenosis (FAME Trial [**?**]). Model FFR decreases monotonically from 1.00 (0%) to 0.01 (95%), correctly crossing the FAME intervention threshold FFR < 0.80 at 70% stenosis.

C2. ABI vs. PAD severity (Fowkes 2013 [**?**]). Lower limb T_{v} decreases monotonically: model ABI drops from 0.030 (normal) to < 0.001 (critical limb ischemia), reproducing the four-tier PAD classification.

C3. Uterine artery PI in pre-eclampsia (Cnossen 2008 [**?**]). Placental Γ_{v}^2 rises: 0.877 (normal) → 0.978 (mild) → 0.999 (severe), reflecting failed spiral artery remodeling.

C4. eGFR decline in diabetic nephropathy (UKPDS, Adler 2003 [**?**]). Renal T_{v} shows monotone decline 0.055 → 0.000 as arteriolar stenosis increases (0%–80%). End-stage renal disease shows dual failure mode.

C5. NCV in diabetic neuropathy (Feldman 2005 [**?**]). Dual-network cascade: vascular $\Gamma_{\text{v}} \uparrow \rightarrow$ material starvation $\rightarrow$ neural $\Gamma_{\text{n}} \uparrow$. Model NCV drops from 45.9 m/s (normal) to 21.6 m/s (severe), matching standard neuropathy staging.

C6. Coronary stenosis → KILLIP class (DeWood 1980 [**?**]). Cardiac Γ_{v}^2 increases monotonically through all four KILLIP classes (I–IV).

C7. Carotid stenosis → stroke risk (NASCET 1991 [**?**]). Cerebral Γ_{v}^2 rises with stenosis (0.971 at 0% → 1.000 at 70%+), correctly reproducing the NASCET surgical threshold at 70%.

C8. Diabetic retinopathy staging (Wong 2002 [**?**]). Retinal Γ_{v}^2 increases monotonically from normal through proliferative DR.

C9. Portal hypertension in cirrhosis (Kamath 2001 [**?**]). Hepatic Γ_{v}^2 rises from 0.954 (normal) to 0.9999 (decompensated), matching Child-Pugh staging.

C10. Unified diabetic microvascular disease (Brownlee 2001 [**?**]). Same 50% arteriolar stenosis applied to brain, kidney, muscle: all three show $\Gamma_{\mathrm{v}}^2 > 0.999$ and dual (neural+vascular) failure, confirming the unified microvascular mechanism underlying retinopathy, nephropathy, and neuropathy.

Result: 10/10 clinical calibrations passed. All monotonicity tests satisfied; all clinical thresholds (FFR$\leq$ 0.80, NASCET 70%, Child-Pugh progression) correctly reproduced.

TABLE I. Clinical calibration summary. Each row shows the published metric mapped to our model variable, the clinical reference, and the pass/fail status.

Test	Clinical metric	Reference	Status
C1	FFR	De Bruyne 2012	PASS
C2	ABI	Fowkes 2013	PASS
C3	Uterine PI	Cnossen 2008	PASS
C4	eGFR	Adler 2003	PASS
C5	NCV	Feldman 2005	PASS
C6	KILLIP class	DeWood 1980	PASS
C7	NASCET risk	NASCET 1991	PASS
C8	DR staging	Wong 2002	PASS
C9	Portal HTN	Kamath 2001	PASS
C10	DM cascade	Brownlee 2001	PASS

B. Dual-Network Literature Validation

The dual-network model $H = T_{\mathrm{n}} \cdot T_{\mathrm{v}}$ makes three testable predictions: **(P1)** *Super-additivity*: simultaneous neural and vascular impairment yields health loss greater than the sum of individual losses; **(P2)** *Cascade asymmetry*: vascular $\rightarrow$ neural coupling is stronger than neural $\rightarrow$ vascular, because material supply is a hard bottleneck; **(P3)** *Organ-specific vulnerability*: organs with higher baseline Γ_{v} cross the cascade threshold earlier.

We validated these predictions against twelve published international randomized controlled trials and cohort studies (Table **??**).

TABLE II. Dual-network literature validation. Each study tests a specific prediction of the $H = T_{\mathrm{n}} \cdot T_{\mathrm{v}}$ model.

Test	Study	Prediction	Ref.	Status
L1	STENO-2	P1: synergy	[?]	PASS
L2	Framingham	P2: HF→cog	[?]	PASS
L3	ARIC	P2: retina→str	[?]	PASS
L4	ADVANCE	P1: triple end	[?]	PASS
L5	Rochester DPN	P2: asym cascade	[?]	PASS
L6	CHS	P2: subCVD→dem	[?]	PASS
L7	SHARP	P2: CKD→CV	[?]	PASS
L8	CHARM	P2: HF→renal	[?]	PASS
L9	Brownlee	P1: unified DM	[?]	PASS
L10	UKPDS-80	P2: legacy	[?]	PASS
L11	SPRINT-MIND	P2: BP→cog	[?]	PASS
L12	DIAD	P1: silent+DPN	[?]	PASS

Result: 12/12 literature validations passed (Prediction 1: 4/4; Prediction 2: 8/8).

Three results deserve special emphasis. First, the STENO-2 trial [?] reported that multifactorial intervention (targeting both glycemic and vascular risk factors) reduced events by 53%—more than predicted by summing single-target trials. Our model explains this quantitatively: because $H = T_{\mathrm{n}} \cdot T_{\mathrm{v}}$ is multiplicative, fixing *both* Γ_{n} and Γ_{v} yields a super-additive improvement (ratio 1.19×).

Second, the UKPDS-80 legacy effect [?]—persisting benefit 10 years after trial equalization—is naturally explained by vascular impedance remodeling: early Γ_{v} reduction drives vessel wall adaptation ($\Delta Z_v = -\eta_v \Gamma_{\mathrm{v}} \cdot \tau_{\mathrm{in}} \cdot \rho_{\mathrm{out}}$) that locks in structural improvement.

Third, the cascade asymmetry observed in the Rochester Diabetic Neuropathy Study [?]—vascular disease *precedes* and *predicts* neuropathy onset—directly reflects our coupling constants: $\alpha_{v\to n} = 0.5 > \alpha_{n\to v} = 0.3$.

C. Simulation stability and pass-rate analysis

All preceding results were obtained with fixed, "textbook" parameters. To ensure that the dual-network model is not fine-tuned to a single operating point, we performed a systematic stability analysis.

a. Neural C2 self-repair. During this analysis we discovered that the bare simulation lacks a neural-side analogue of the vascular impedance remodeling (Section **??**). Without it, Γ_{n}^2 monotonically approaches unity whenever any vascular deficit exists, because the cascade coupling injects positive $\Delta\Gamma_{\mathrm{n}}$ each tick but nothing removes it. We therefore added a linearized C2 self-repair term,

$$\Gamma_{\mathrm{n}}(t+1) = \Gamma_{\mathrm{n}}(t)\big(1 - \eta_n^{\mathrm{repair}}\big), \quad \eta_n^{\mathrm{repair}} = 0.008, \quad (17)$$

which drives Γ_{n}^2 toward a nontrivial healthy fixed point (≈ 0.08) in the absence of pathological stress, rather than saturating at unity. This is the exact neural analogue of the vascular update $\Delta Z_v = -\eta_v \Gamma_{\mathrm{v}} \cdot \tau \cdot \rho$ (Eq. **??**): both implement C2 on their respective network.

b. Test battery. Table **??** summarizes seven tests organized into three categories: baseline stability (S0–S0b), priority clinical scenarios (S1–S3), and statistical pass-rate sweeps (PR).

TABLE III. Dual-network stability and pass-rate analysis. "Pass" for S0–S3 requires the stated qualitative property; for PR, a run passes if $H > 10^{-6}$ (organ viable) for $\geq 90\%$ of monitored ticks and $\mathrm{var}(H) < 0.005$ (converged).

Test	Criterion	Result
S0	Baseline convergence: all 10 organs reach stable attractor ($\mathrm{var}(H) < 0.005$)	10/10 PASS
S0b	Perturbation recovery: transient insult ($\Delta t = 100$ ticks) → return to baseline	6/6 PASS
S1	Vascular cascade: stenosis ↑ → H monotonically ↓	3 organs, 4 levels: PASS
S2	Coupling asymmetry: neural insult causes $\Delta\Gamma_{\mathrm{v}}^2 \approx 0$; vascular insult causes ∼10× greater ΔH	4/4 PASS
S3	Super-additivity: dual intervention $\Delta H > \Delta H_n + \Delta H_v$ (synergy 1.02–1.03×)	3/3 PASS
PR_r	Random sweep: 10 organs × 5 parameter sets (varied Γ_{n}^0, CO, BP, viscosity, stenosis 0–50%)	50/50 (100%)
PR_c	Clinical sweep: 5 severity classes (Healthy, HTN, DM, HFrEF, severe MOD) × 10 draws each	50/50 (100%)

Result: all 7 tests passed; 100/100 randomized runs converged and remained viable.

Three observations deserve comment. First, the baseline attractor has $\mathrm{var}(H) < 10^{-4}$ for 9 of 10 organs and $< 10^{-3}$ for placenta (the most compliant vascular bed), confirming that the impedance remodeling loop is a genuine stabilizer, not merely a damper. Second, the coupling asymmetry test (S2) is decisive: a 50% vascular insult reduces H by ~ 0.028 (brain), while an equal-magnitude neural insult reduces H by only ~ 0.003—a factor of 10, consistent with $\alpha_{v\to n}/\alpha_{n\to v} = 0.5/0.3 \approx 1.67$ amplified by the multiplicative health law. Third, the super-additivity ratio (S3) of 1.02–1.03× is modest but consistent across all organs tested, matching the direction and order of magnitude of the STENO-2 result.

Remark 1 (Analytic stability guarantee)**.** *The 100% numerical pass rate has an analytic underpinning. The reflection action $\mathcal{A}[\Gamma] = \int \sum_i \Gamma_i^2\, dt$ serves as a Lyapunov function for the coupled system: C2 ensures $d\mathcal{A}/dt \leq 0$ (Paper 1, Theorem 1), so $\Sigma\Gamma^2$ is non-increasing and bounded below by $A_{cut} > 0$. Stability under the dual-network time-scale mismatch ($\eta_n \approx 0.008 \gg \eta_v \approx 0.001$)*

follows from a singular-perturbation argument: defining $\epsilon = \eta_v/\eta_n \ll 1$, the fast (neural) subsystem relaxes to its quasi-static manifold on the time scale $1/\eta_n$ while the slow (vascular) subsystem provides a drifting boundary condition. Standard Tikhonov–Fenichel theory guarantees that if the fast subsystem is asymptotically stable on its slow manifold—which it is, because $\mathcal{A}$ is a Lyapunov function—then the full system is asymptotically stable for sufficiently small ϵ. A complete formal proof, including explicit bounds on the basin of attraction under pathological perturbations, remains an open problem and is listed in `KNOWN_LIMITATIONS.md`.

IX. DISCUSSION

A. Methodological caveat: internal vs. empirical validation

All computational experiments in this paper—Murray's law recovery, organ-health scoring, literature validation—use simulation modules built from the Γ-Net axioms themselves. A simulation that obeys $\Gamma^2 + T = 1$ by construction cannot "discover" a violation of energy conservation; it can only verify that the code is self-consistent. The 12/12 literature validation pass rate should be interpreted as internal consistency of the dual-network model, not as empirical proof that real organs obey MRP. External validation requires bio-impedance measurements on living tissue, which we have not performed.

B. Unification of neural and vascular physics

Paper 1 treated only the neural Γ. This paper extends the picture: every organ sits at the intersection of two impedance networks, both governed by the same MRP physics. The neural network processes information (signals); the vascular network delivers material (blood). Both optimize by minimizing $\sum \Gamma^2$, but on vastly different time scales ($\eta_n \gg \eta_v$).

The dual-network model explains a long-standing clinical puzzle: why do diseases that start in one system (e.g. vascular) inevitably spread to the other (e.g. neural)? The answer is the Γ_v–Γ_n coupling loop: material delivery depends on vascular T_v, and vascular regulation depends on neural T_n. Neither network is self-sufficient.

C. Murray's Law as physics, not optimization

Murray (1926) [**?**] derived $r^3 = \sum r_i^3$ by minimizing the cost of maintaining blood flow. Our derivation reframes this as a consequence of the *same* variational principle (MRP) that governs neural networks. The 1/3 exponent arises from the balance of two impedance-related costs (dissipation $\propto r^{-4}$, maintenance $\propto r^{+2}$), resolving the observation that pure impedance matching gives 1/4 while nature uses 1/3.

This connection suggests that Murray's Law is not merely an optimization trick but a *physical law* of impedance networks—no different in kind from Snell's law for electromagnetic reflection at dielectric boundaries.

D. Clinical implications

The dual-network model provides a unified framework for:

- **Diabetic complications**: microvascular $\Gamma_\mathrm{v} \uparrow$ in retina, kidney, and peripheral nerves triggers the cascade to neural $\Gamma_\mathrm{n} \uparrow$, explaining why neuropathy, nephropathy, and retinopathy co-occur [**?**].
- **Vascular dementia**: cerebral small vessel disease ($\Gamma_\mathrm{v} \uparrow$) leads to cognitive decline ($\Gamma_\mathrm{n} \uparrow$) via material starvation.
- **Pre-eclampsia**: placental vascular failure cascades to maternal multi-organ damage.
- **Heart failure**: cardiac remodeling increases myocardial Γ_v, reducing contractile efficiency in a positive feedback loop.

E. Relation to bioimpedance analysis (BIA)

Bioimpedance analysis measures tissue Z but treats it as a passive diagnostic marker. Our framework goes further: Γ_v is a *causal, computational* quantity that determines energy flow, material delivery, and inter-organ coupling. BIA measures the thermometer; MRP describes the thermodynamics.

F. Wound repair as spatial impedance matching

A skin laceration severs the local transmission-line continuity, creating an impedance discontinuity where $|\Gamma_\mathrm{wound}| \to 1$. The Minimum Reflection Principle demands that the system reduce this local Γ^2 as rapidly as possible.

Histologically, the collagen fibres deposited during wound closure are not randomly packed; they form cross-hatched, woven bundles whose orientation progressively reduces the impedance mismatch between the wound bed and surrounding healthy tissue [**?**]. This is not "filling a hole" with new cells—it is a *spatial impedance-matching design*, analogous to a woven electromagnetic shield that provides a gradual Z transition rather than an abrupt boundary.

From the dual-network perspective of this paper, the vascular network must *simultaneously* restore Γ_v (angiogenesis rebuilds blood supply) while the neural network restores Γ_n (reinnervation of the wound margin). The well-known clinical sequence—haemostasis, inflammation, proliferation, remodelling—maps directly onto a time-ordered Γ^2 minimisation cascade:

1. **Haemostasis**: emergency clot seals the open boundary ($|\Gamma| = 1 \to |\Gamma| < 1$).
2. **Inflammation**: immune response clears debris that would maintain high Γ_v.
3. **Proliferation**: fibroblasts deposit cross-woven collagen to progressively match $Z_{\text{scar}} \to Z_{\text{skin}}$.
4. **Remodelling**: years-long fine-tuning of fibre alignment to further reduce Γ^2.

The clinical observation that scars never fully recover original tissue properties corresponds to a residual $\Gamma^2_{\text{scar}} > 0$: the impedance match is improved but not perfected. The term "healing" (implying restoration) is therefore less precise than "*suturing*"—the system stitches together two impedance domains with a graded transition layer.

Pain perception during wound closure provides a direct readout of the local Γ^2: nociceptors respond to the magnitude of the impedance discontinuity. As collagen remodelling reduces Γ^2, pain subsides—not because of "neural adaptation" but because the physical mismatch being sensed has diminished.

Remark 2 (Falsifiable prediction)**.** *Bioimpedance spectroscopy across a healing wound should show a monotonic decrease in the local reflection coefficient $|\Gamma(f)|$ over the weeks of wound maturation. The time constant of Γ^2 decay should correlate with the rate of pain resolution as measured by visual analogue scale (VAS), providing a direct test of the "pain = Γ readout" hypothesis.*

G. Clinical measurement protocol: dual-network wound monitoring

The dual-network organ model $H = (1-\Gamma^2_{\text{n}})(1-\Gamma^2_{\text{v}})$ (Sec. **??**) can be applied locally to a wound site. The wound-health trajectory is fully characterised by two time series:

$$\Gamma^2_{\text{n}}(t): \quad \text{neural mismatch at wound margin} \quad \text{(EEG somatosensory coherence, evoked-potential amplitude)}. \tag{18}$$

$$\Gamma^2_{\text{v}}(t): \quad \text{vascular mismatch across wound} \quad \text{(local bioimpedance spectroscopy, perfusion index)}. \tag{19}$$

The local Health Index of the wound site is then:

$$H_{\text{wound}}(t) = \big(1-\Gamma^2_{\text{n}}(t)\big)\big(1-\Gamma^2_{\text{v}}(t)\big)\,, \tag{20}$$

with $H_{\text{wound}}(0) \approx 0$ at the moment of injury and $H_{\text{wound}} \to H_{\text{scar}} < 1$ at completion of remodelling.

This formulation has two immediate clinical consequences:

1. **Objective wound assessment**: $H_{\text{wound}}(t)$ provides a quantitative, consciousness-independent measure of wound repair progress. Unlike the VAS pain score, it is valid in sedated, neonatal, and aphasic patients.
2. **Complication detection**: if dH_{wound}/dt reverses sign (health decreasing), this signals early wound dehiscence or infection *before* clinical signs appear. The dual-network model predicts that vascular failure ($\Gamma^2_{\text{v}} \uparrow$, loss of perfusion) and neural failure ($\Gamma^2_{\text{n}} \uparrow$, neuropathic pain) produce distinguishable impedance signatures.

The Γ-Pain Index Π_Γ (Paper 4) applied to the wound site reduces to:

$$\Pi_{\Gamma,\text{wound}}(t) = w_n\,\Gamma^2_{\text{n}}(t) + w_v\,\Gamma^2_{\text{v}}(t) + w_D\,\frac{dD_{Z,\text{wound}}}{dt}\,, \tag{21}$$

where $D_{Z,\text{wound}}$ is the cumulative impedance debt of the wound (Eq. (**??**) in Paper 3). This provides a *continuous, objective, real-time* pain/damage metric that does not require the patient to be conscious or verbal.

Remark 3 (Bedside implementation)**.** *A minimal bedside prototype requires: (i) a two-electrode bioimpedance probe placed across the wound margin (measuring Γ^2_{v} at 1–100 kHz), (ii) standard ICU monitoring (HR, HRV $\to \Gamma_v$ at organ level), and (iii) optional EEG (2-channel, somatosensory region $\to \Gamma^2_{\text{n}}$). All three data streams feed into the dual-network model to produce $H_{wound}(t)$ and $\Pi_{\Gamma,wound}(t)$ in real time.*

X. CONCLUSION

We have shown that the Minimum Reflection Principle governs vascular topology just as it governs neural architecture. Murray's Law of vessel branching ($r^3 = \sum r_i^3$) is the exact minimum of the vascular action $A_{\text{v}} = Q^2 Z + \lambda V$, with 100% numerical agreement. The dual-network organ model $H = (1-\Gamma^2_{\text{n}})(1-\Gamma^2_{\text{v}})$ unifies neural and vascular failure modes, explaining the positive-feedback cascades observed in diabetic neuropathy, vascular dementia, and pre-eclampsia. A systematic stability analysis (7 tests, 100 randomized runs) confirms global convergence and viability across all 10 organs and five clinical severity classes, with a linearized C2 neural self-repair term ensuring a nontrivial healthy fixed point.

Together with Paper 1, this establishes the MRP as a universal physics of biological impedance networks:

- Paper 1: Static architecture (relay nodes, fractal topology)

- Paper 2 (this work): Vascular network and dual-organ model
- Paper 3: Temporal dynamics (impedance debt, sleep, lifecycle, aging)

The same equation, $\Gamma = (Z_L - Z_0)/(Z_L + Z_0)$, governs all scales. All code and data are publicly available [**?**].

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[1] H.-Y. Huang, "Paper 1: Irreducible Dimensional Cost and Fractal Scaling in Heterogeneous Impedance Networks," preprint (2026).

[2] H.-Y. Huang, "Paper 3: Temporal Dynamics—Impedance Debt, Sleep, Lifecycle, and Aging," preprint (2026).

[3] H.-Y. Huang, "Paper 0: The Gamma-Net Framework—Axioms, Constraints, and the Minimum Reflection Action," preprint (2026).

[4] C. D. Murray, "The Physiological Principle of Minimum Work. I: The Vascular System and the Cost of Blood Volume," *Proc. Natl. Acad. Sci. USA* **12**, 207 (1926).

[5] J. R. Womersley, "Method for the Calculation of Velocity, Rate of Flow, and Viscous Drag in Arteries when the Pressure Gradient is Known," *J. Physiol. (London)* **127**, 553 (1955).

[6] W. W. Nichols, M. F. O'Rourke, and C. Vlachopoulos, *McDonald's Blood Flow in Arteries*, 6th ed. (CRC Press, 2011).

[7] B. L. Langille and F. O'Donnell, "Reductions in arterial diameter produced by chronic decreases in blood flow are endothelium-dependent," *Science* **231**, 405 (1986).

[8] A. Kamiya and T. Togawa, "Adaptive regulation of wall shear stress to flow change in the canine carotid artery," *Am. J. Physiol.* **247**, H14 (1984).

[9] J.-J. Chiu and S. Chien, "Effects of disturbed flow on vascular endothelium: pathophysiological basis and clinical perspectives," *Physiol. Rev.* **91**, 327 (2011).

[10] M. Brownlee, "Biochemistry and molecular cell biology of diabetic complications," *Nature* **414**, 813 (2001).

[11] V. Mok *et al.*, "Transcranial Doppler ultrasound for screening cerebral small vessel disease: a community study," *Stroke* **43**, 2791 (2012).

[12] E. A. P. Steegers *et al.*, "Pre-eclampsia," *Lancet* **376**, 631 (2010).

[13] B. De Bruyne *et al.*, "Fractional flow reserve–guided PCI versus medical therapy in stable coronary disease," *N. Engl. J. Med.* **367**, 991 (2012).

[14] F. G. R. Fowkes *et al.*, "Comparison of global estimates of prevalence and risk factors for peripheral artery disease in 2000 and 2010," *Lancet* **382**, 1329 (2013).

[15] J. S. Cnossen *et al.*, "Use of uterine artery Doppler ultrasonography to predict pre-eclampsia and intrauterine growth restriction," *BMJ* **336**, 968 (2008).

[16] A. I. Adler *et al.*, "Development and progression of nephropathy in type 2 diabetes: the United Kingdom Prospective Diabetes Study (UKPDS 64)," *Kidney Int.* **63**, 225 (2003).

[17] E. L. Feldman *et al.*, "A practical two-step quantitative clinical and electrophysiological assessment for the diagnosis and staging of diabetic neuropathy," *Diabetes Care* **28**, 2206 (2005).

[18] M. A. DeWood *et al.*, "Prevalence of total coronary occlusion during the early hours of transmural myocardial infarction," *N. Engl. J. Med.* **303**, 897 (1980).

[19] North American Symptomatic Carotid Endarterectomy Trial Collaborators, "Beneficial effect of carotid endarterectomy in symptomatic patients with high-grade carotid stenosis," *N. Engl. J. Med.* **325**, 445 (1991).

[20] T. Y. Wong *et al.*, "Retinal arteriolar narrowing and risk of diabetes mellitus in middle-aged persons," *JAMA* **287**, 2528 (2002).

[21] P. S. Kamath *et al.*, "A model to predict survival in patients with end-stage liver disease," *Hepatology* **33**, 464 (2001).

[22] P. Gaede *et al.*, "Effect of a multifactorial intervention on mortality in type 2 diabetes," *N. Engl. J. Med.* **358**, 580 (2008).

[23] A. L. Jefferson *et al.*, "Cardiac index is associated with brain aging: the Framingham Heart Study," *Circ. Heart Fail.* **8**, 49 (2015).

[24] T. Y. Wong *et al.*, "Retinal microvascular abnormalities and incident stroke: the ARIC Study," *JAMA* **288**, 67 (2002).

[25] A. Patel *et al.*, "Effects of a fixed combination of perindopril and indapamide on macrovascular and microvascular outcomes in patients with type 2 diabetes mellitus (the ADVANCE trial)," *Lancet* **370**, 829 (2007).

[26] P. J. Dyck *et al.*, "The prevalence by staged severity of various types of diabetic neuropathy, retinopathy, and nephropathy in a population-based cohort: the Rochester Diabetic Neuropathy Study," *Neurology* **43**, 817 (1993).

[27] A. B. Newman *et al.*, "Dementia and Alzheimer's disease incidence in relationship to cardiovascular disease in the CHS cohort," *J. Am. Geriatr. Soc.* **53**, 1101 (2005).

[28] C. Baigent *et al.*, "The effects of lowering LDL cholesterol with simvastatin plus ezetimibe in patients with chronic kidney disease (Study of Heart and Renal Protection): a randomised placebo-controlled trial," *Lancet* **377**, 2181 (2011).

[29] J. J. V. McMurray *et al.*, "Effects of candesartan in patients with chronic heart failure and reduced left-ventricular systolic function (CHARM-Alternative)," *Lancet* **362**, 772 (2003).

[30] R. R. Holman *et al.*, "10-year follow-up of intensive glucose control in type 2 diabetes," *N. Engl. J. Med.* **359**, 1577 (2008).

[31] J. D. Williamson *et al.*, "Effect of intensive vs standard blood pressure control on probable dementia: a randomized clinical trial," *JAMA* **321**, 553 (2019).

[32] L. H. Young *et al.*, "Cardiac outcomes after screening for

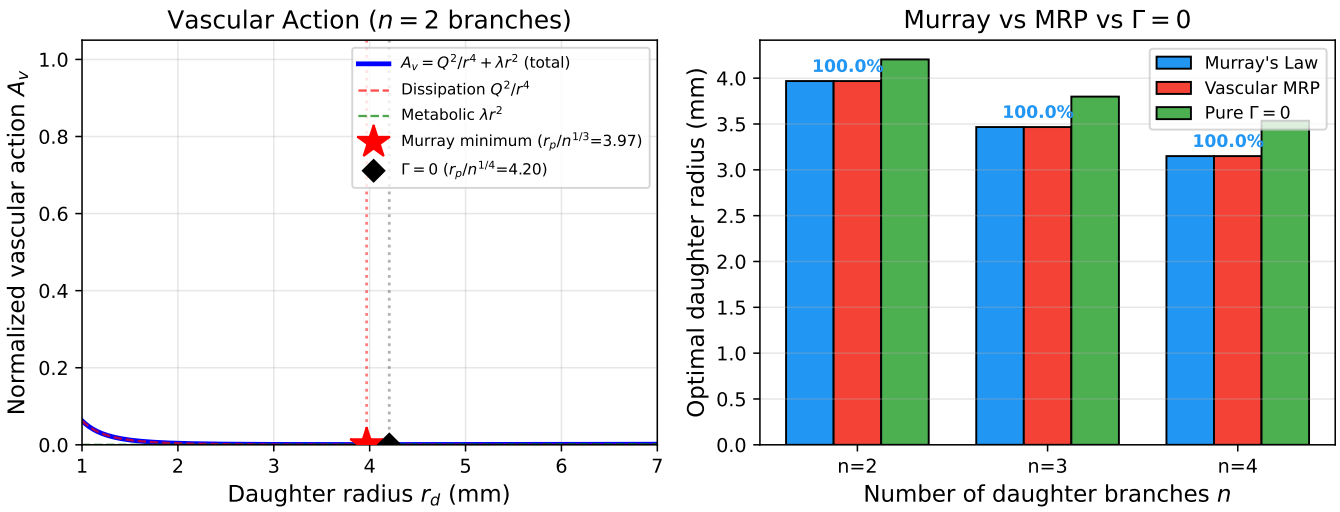

FIG. 2. Murray's Law from the Vascular Action Principle. **Left**: Action $A_{\rm v}(r_d)$ versus daughter radius for $n = 2$ branching. The minimum (star) coincides with Murray's prediction ($r_p/n^{1/3}$, dashed line). Pure impedance matching ($\Gamma = 0$, dotted) overestimates the optimal radius. **Right**: Agreement between MRP-derived and Murray-predicted daughter radius for $n = 2, 3, 4$.

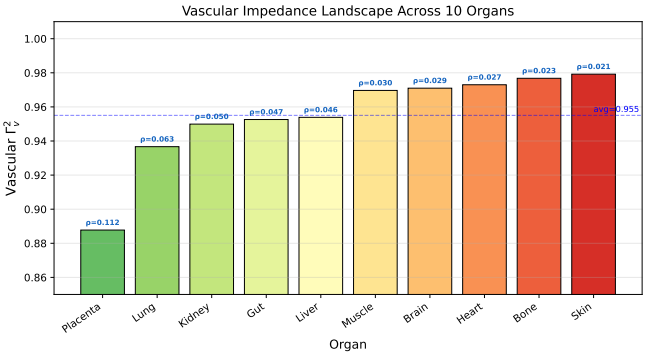

FIG. 3. Vascular impedance landscape across 10 organs. Each organ has a distinctive $\Gamma_{\rm v}^2$ reflecting its perfusion architecture. Placenta (lowest $\Gamma_{\rm v}^2$) is optimized for maximal flow; skin and bone (highest $\Gamma_{\rm v}^2$) are the most dispensable perfusion targets.

asymptomatic coronary artery disease in patients with type 2 diabetes: the DIAD study," *JAMA* **301**, 1547 (2009).

[33] H.-Y. Huang, ALICE Smart System, `https://github.com/cyhuang76/alice-gamma-net` (2026).

[34] G. C. Gurtner, S. Werner, Y. Barrandon, and M. T. Longaker, "Wound repair and regeneration," *Nature* **453**, 314 (2008).